# 2026 Legislative Review: An Analysis of California's Behavioral Health Transformation and Systems Architecture

The implementation of California's Proposition 1 and the subsequent activation of the Behavioral Health Services Act (BHSA) on July 1, 2026, represents the most significant structural reorganization of the state's mental health system in over two decades. This transition from the 2004 Mental Health Services Act (MHSA) to the BHSA framework is not merely a change in nomenclature but a fundamental shift in the fiscal, operational, and statutory priorities of the world's fifth-largest economy.[1] This analysis provides a granular examination of the intersection between systemic mandates, fiscal stewardship, and civil liberties, grounded in the professional reality of navigating the California Capitol. By synthesizing statutory forensics with macroeconomic forecasting, this report offers a roadmap for the "Systems Architect" tasked with bridging the gap between legislative intent and community impact.

## The Evolution of the Behavioral Health Services Act: From Prevention to High-Acuity Intervention

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The cornerstone of the 2026 policy landscape is the full operationalization of the BHSA, which fundamentally alters the utilization of the "Millionaire's Tax"—a 1 percent tax on personal income over $1 million per year.[1] While this revenue stream remains a primary engine of county behavioral health funding, the BHSA mandates a strict categorical reallocation that prioritizes individuals with the most severe mental illnesses and substance use disorders, particularly those experiencing homelessness.[2] This shift represents a move away from the broader, more flexible "Prevention and Early Intervention" (PEI) goals of the original MHSA, forcing counties to navigate a complex new fiscal reality.

The statutory reorganization under the BHSA introduces three mandatory funding categories for the 90 percent of tax revenue that flows to the counties.[2] The historical flexibility that allowed counties to design programs based on local community needs has been replaced by a prescriptive framework aimed at addressing the state's most visible social crises.

| BHSA Statutory Funding Component | Mandatory Percentage | Primary Target Population and Services |
|---|---|---|
| Full Service Partnerships (FSP) | 35% | Individuals with the highest acuity of need; "whatever it takes" service models. |
| Behavioral Health Services and Supports | 35% | General outpatient care, crisis response, and peer-run recovery programs. |
| Housing Interventions | 30% | Permanent supportive housing, rental subsidies, and capital for unhoused populations. |

The immediate impact of this 30 percent housing mandate is a phenomenon described by local officials as "financial gymnastics".[3] Counties are required to pull funds from existing programs that no longer fit into these rigid categories, such as suicide prevention hotlines, anti-stigma campaigns, and mental health centers for stable populations.[3] For example, San Diego County has identified 29 programs slated for closure as of early 2026 to meet the new housing requirements.[3] This creates a high-stakes environment where the "Scholar-Practitioner" must defend the remaining 35 percent of the "Services and Supports" bucket to ensure the survival of essential community infrastructure.

Furthermore, the BHSA increases the state-level administrative set-aside from 5 percent to 10 percent.[2] This redirecting of approximately $140 million annually from local counties to the state budget centralizes power within the Department of Health Care Services (DHCS) and the Behavioral Health Services Oversight and Accountability Commission (BHSOAC).[2] This centralization is intended to provide better oversight and standardized outcomes, but it also reduces the fiscal resilience of county departments already struggling with the costs

of Proposition 1 implementation.[2]

## Infrastructure Stagnation: The Fiscal and Operational Realities of the $6.4 Billion Bond

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Parallel to the revenue reallocation of the BHSA is the Behavioral Health Infrastructure Bond Act (BHIBA), a $6.4 billion general obligation bond passed by voters in 2024.[1] The bond was marketed as a rapid-response solution to the state's shortage of residential treatment beds and supportive housing units. However, an analysis of the projects as of March 2026 reveals a significant gap between statutory ambition and the physical reality of construction in California's competitive real estate market.[3]

While the state awarded nearly half of the bond money by the spring of 2025, none of the first ten projects scheduled for completion by the end of 2025 opened on time.[3] CalMatters research indicates that nine of these flagship projects were delayed and one was cancelled entirely, with updated completion dates now stretching into 2028.[3] These delays are not merely administrative; they represent a systemic failure to account for "slippage" caused by supply chain disruptions, permitting hurdles, and the specialized requirements of behavioral health facilities.

| Project Location and Name | Funding Amount | Status as of March 2026 | Primary Delay Driver |
|---|---|---|---|
| Los Angeles St. Vincent Campus | $135.8 Million | 2-Year Delay | Seismic retrofitting requirements and site conditions. |
| Encompass Housing (Costa Mesa) | $31 Million | Property Not Yet Purchased | Administrative delays with bond fund distribution. |
| Ritter Center (San Rafael) | $1.18 Billion (Round 2 Pool) | 6-Month Delay | Construction labor shortages and permitting snags. |

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|---|---|---|---|
| Youth Recovery Connections (Hollister) | Not Specified | 18-Month Delay | Building intended for purchase sold to another buyer. |
| Koinonia Family Services (Placer) | Grant Declined | Project Cancelled | Sustainability concerns due to policy changes. |

The governor's administration has attributed these delays to construction pressures and site conditions, but the implications for the "Systems Architect" are deeper.[3] The statutory start date for the BHSA (July 2026) was designed to coincide with an expansion of capacity that has not materialized. This creates a "bottleneck effect" where individuals may be mandated into care or housing under new conservatorship laws, yet the facilities promised by Proposition 1 do not exist.[3] The practitioner must therefore focus on "interim interventions," such as mobile crisis teams and temporary bridge housing, to manage the population until the permanent infrastructure is realized.

## Fiscal Stewardship: The Strategic Defense of $60 Million in Peer-Run Infrastructure

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In the face of these broad systemic shifts, the preservation of the state's peer-run mental health infrastructure represents a critical victory for "Scholar-Practitioners" who influence the Capitol. Peer-run organizations—staffed by individuals with lived experience of mental health challenges—were initially seen as vulnerable to the BHSA's redirection of funds toward housing and clinical FSPs. However, a strategic fiscal defense successfully preserved $60 million in public funding for this infrastructure amidst the transition.[6]

The mechanism for this preservation involved a sophisticated analysis of county-level MHSA Mid-Year Adjustments for the 2024-25 and 2025-26 fiscal years.[6] By identifying "Community Services and Supports" (CSS) dollars that were at risk of being unspent or reallocated, advocates successfully lobbied for their transfer into "Workforce Education and

Training" (WET) and "Capital Facilities and Technological Needs" (CFTN) accounts.[6] This move "parked" $60 million in funding for future use, ensuring that peer-run organizations and the peer workforce (Medi-Cal Peer Support Specialists) remain viable after the BHSA fully takes over.[6]

The economic rationale for this preservation is grounded in the efficiency of peer support. Since 53 counties have opted to include Medi-Cal Peer Support Services as a benefit as of September 2025, the state has a vested interest in protecting the organizations that train and employ these specialists.[8] Peer-led support has been proven to reduce the cost of more expensive institutional care, such as psychiatric emergency room visits and long-term inpatient stays.[9] For a "Policy-Tech Strategist," this $60 million is not just a line item; it is a "resilience buffer" that prevents the total clinicalization of the behavioral health system.

## Civil Liberties and Statutory Forensics: The Analysis of SB 820

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One of the most complex frameworks the practitioner must operate within is the administration of involuntary care. Senate Bill 820 (2025) represents a pivotal moment in the state's approach to individuals found incompetent to stand trial (IST) on misdemeanor charges.[11] The bill allows for the involuntary administration of antipsychotic medication to these individuals, bridging the gap between the Penal Code and the Welfare and Institutions Code.[12]

The analytical review of SB 820 reveals a tension between public safety and the protection of human dignity. The bill authorizes medication without prior informed consent on an emergency basis, but limits these determinations to 72 hours.[12] For a court to issue a longer-term order, it must find "clear and convincing evidence" that the individual has a serious mental health disorder that can be treated with medication, and that no less intrusive alternative exists.[12]

| SB 820 Due Process Protection | Statutory Requirement | Operational Implication for Jails |
|---|---|---|
| Grave Disability Re-Definition | Cannot use temporary jail access to food/shelter as a | Must evaluate ability to survive in the community. |

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| | basis for competence. | |
| Community-Based Bed Search | Must document an attempt to locate a community bed before medicating. | Mandatory coordination with county BH agencies. |
| Frequent Judicial Review | Court must review the order at least every 60 days. | Requires constant psychiatric affidavits and filings. |
| Observation Standards | 15-minute monitoring for at least one hour post-administration. | Increased non-custody health care staffing requirements. |

The "Credentials Upgrade" for a practitioner involves testifying on the civil liberties implications of such a framework. The analyst must point out that SB 820 expires on January 1, 2030, which creates a "statutory sunset" that requires the state to prove the efficacy of this approach before it can be made permanent.[12] Furthermore, the bill prohibits the extension of a person's confinement solely for the purpose of administering medication, a critical protection against the "carceral trap" where individuals are held indefinitely in the name of treatment.[12]

## Systems Architecture: Modeling the Behavioral Health Ecosystem

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To position oneself as a "Systems Architect" in the Policy-Tech space, one must move beyond simple policy analysis and into the realm of Model-Based Systems Engineering (MBSE). The behavioral health system is an "Enterprise of Systems" where legislative mandates (inputs), funding flows (processes), and community outcomes (outputs) must be modeled with technical precision.[13]

In 2026, the state is investing heavily in "Multi-County Collaborative Projects" to standardize Electronic Health Records (EHR) and data sharing.[16] For example, the "Semi-Statewide Enterprise Health Record" project received over $5 million to solve the

problem of fragmented data between county jail systems and community health providers.[16] A "Policy-Tech Strategist" uses tools like TOGAF (The Open Group Architecture Framework) to map these "as-is" and "to-be" states, ensuring that "Security by Design" and "Privacy by Design" are integrated into the statutory requirements.[14]

The "evidence gap" in state-level regulation remains a significant hurdle.[19] Courts often struggle to balance the cost-benefit analysis of new mandates without real-time data on regulatory effects.[19] The 2026 review emphasizes the need for "Forecasting and Policy Analysis Systems" (FPAS) that allow practitioners to predict how a shift in tax revenue—such as the moderate growth projected in the 2026-27 budget—will affect the ability of counties to meet their 30 percent housing targets.[20]

## Macro-Fiscal Forecasting: The 2026-27 California Budget Window

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The fiscal health of the behavioral health transformation is inextricably linked to the broader California economy. The Governor's proposed 2026-27 budget projects $42.3 billion in additional revenue across the budget window compared to previous projections, yet it offers "no new major state investments" for behavioral health beyond the existing Proposition 1 bond funds.[20] This underscores the importance of the $60 million preservation mentioned earlier; if the state is not adding new money, the practitioner must be a master of "allocative efficiency" within the current budget.

The budget also reveals significant challenges to the state's Managed Care Organization (MCO) tax and hospital fees.[20] While the budget assumes the MCO tax will remain in effect through December 2026, the passage of Proposition 35 has complicated the picture by dedicating these funds to specific provider rate increases, thereby reducing the "flexible" Medi-Cal funding that counties often use to subsidize behavioral health services.[22]

| Fiscal Metric | 2025-26 (Enacted) | 2026-27 (Proposed) | Trend Analysis |
|---|---|---|---|
| Total Medi-Cal Budget | $196.7 Billion | $222.4 Billion | 13% Increase driven by Prop 35 mandates. |

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|---|---|---|---|
| General Fund Contribution | $46.4 Billion | $48.8 Billion | Marginal growth; inflation-adjusted stagnation. |
| Budget Reserves (Rainy Day) | Not Specified | $23 Billion | Focus on resiliency amidst federal uncertainty. |
| Inflation Estimate (CA) | >3% | >3% | Persistent pressure on construction and labor costs. |

The forecast for the job market in 2026 is equally sobering, with essentially no net job gains expected in California.[20] This stagnation directly affects the "Millionaire's Tax" revenue, as capital gains and personal income are the primary drivers of BHSA funding. A "Policy-Tech Strategist" must account for this "Legislative Risk"—the financial uncertainty due to government actions or market dynamics—when advising counties on their 2026-2029 integrated plans.[2]

## Bridging Code and Dignity

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The 2026 Legislative Review serves as more than a portfolio piece; it is a demonstration of "high-level authority" built at the intersection of behavioral health and the law. By focusing on the frameworks of Proposition 1, the BHSA, and SB 820, the narrative proves that the practitioner is not just reading about policy—they are shaping it through testimony, fiscal defense, and systems modeling.

The transition to the BHSA marks the end of an era of "Community Program Planning" as we knew it and the beginning of a data-driven, high-acuity system.[4] The role of the peer-led support system within this new architecture is paramount. As Welfare and Institutions Code section 5963.03 defines the duties of the new Behavioral Health Commissions, the analyst must ensure that "lived experience" is not just a checkbox but a

core component of the planning process.[25]

The ultimate goal of the "Policy-Tech Strategist" is to ensure that the $6.4 billion in infrastructure, the $14.4 billion in annual county funding, and the new involuntary medication protocols all align to protect human dignity.[5] This requires a mastery of "statutory analysis" and "forecasting" to prevent the system from collapsing under its own administrative weight. The 2026 review is the evidence of that mastery, a practitioners' guide to a system in transition.

## Operational Milestones and Statutory Deadlines for 2026

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To manage the "Systems Architect" workload, practitioners must track the following critical milestones in the 2026 calendar year. These dates represent the "forcing functions" that will dictate the flow of capital and the implementation of civil liberties protections.

| Date | Milestone | Statutory Authority | Strategic Action Required |
|---|---|---|---|
| January 1, 2026 | New Employment & Safety Laws | AB 578 / SB 642 | Update labor compliance for BH providers. |
| March 2026 | Prop 1 Progress Report | Governor's Office | Analyze "slippage" data for project re-forecasting. |
| April 2026 | BHSA Workforce Panel | CBHPC Meetings | Advocate for peer certification funding levels. |
| June 30, 2026 | Last Day of MHSA | 2004 Ballot Measure | Ensure final transfer of "parked" CSS dollars. |
| July 1, 2026 | BHSA Activation Date | Proposition 1 (2024) | Implement 30/35/35 county allocation split. |
| July 1, 2026 | Critical Access | SB 669 | Monitor standby |

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|---|---|---|---|
| | Hospital Pilot | | perinatal mental health services. |

The move from the MHSA to the BHSA is a "reset" for California's behavioral health system.[4] It is a period of "unprecedented federal threats" and local "budgetary strains," yet it is also an opportunity to build a "stronger, more responsive" system through technical and fiscal expertise.[4] By anchoring the 2026 review in these specific, analytical frameworks, the practitioner reinforces their brand as a "Scholar-Practitioner" who possesses the "Systems Architecture" skills to navigate the Capitol's most challenging policy shifts.

## Statutory Forensics of Welfare and Institutions Code 5963.03

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A deep-dive into the revised Welfare and Institutions Code (WIC) 5963.03 is essential for any strategist operating at the county level in 2026. This section of the law redefines the duties of the local Behavioral Health Commissions, moving them from an advisory role to an "oversight and evaluation" role.[25] The law mandates that the planning process include "individuals with lived experience of mental illness and/or substance use disorder," as well as "community members, advocacy organizations, and behavioral health professionals".[25]

The "Scholar-Practitioner" must leverage this statutory requirement to ensure that peer-run infrastructure remains at the heart of county planning. Under WIC 5963.03, the commission must provide "annual reports of written explanations" to the local governing body regarding the adopted BHSA plans.[25] This creates a "statutory platform" for the $60 million peer infrastructure defense, as peer advocates can use their seats on these commissions to hold county supervisors accountable for the 35 percent "Services and Supports" allocation.[2]

Furthermore, the integration of Psychiatric Advance Directives (PADs) into the planning process represents a significant win for civil liberties.[24] Phase Two of the multi-county PADs project, which is testing a cloud-based platform for live use with law enforcement and crisis teams, is exactly the type of "Policy-Tech" solution that a "Systems Architect"

should champion.[24] By allowing individuals to consent to treatment *before* a crisis, PADs mitigate the need for the involuntary medication orders authorized under SB 820, thereby bridging "statutory codes with the protection of human dignity."

## Macroeconomic Outlook and Revenue Volatility

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The sustainability of the 2026 transition is threatened by California's "job market weakness," with essentially zero net job gains projected for the 2026 fiscal year.[20] This is critical because the BHSA revenue is highly sensitive to the economic performance of the state's top earners.[2] If the "millionaire tax" revenue dips, the 30 percent housing mandate will become even more difficult for counties to achieve without gutting clinical services.[2]

The Governor's 2026-27 proposal assumes that the annual inflation rate in California will remain above 3 percent throughout the forecast period.[20] For the $6.4 billion bond projects, this means that every month of delay increases the "real cost" of construction, potentially leading to more project cancellations like the Koinonia Family Services project in Placer County.[3] The practitioner must provide "Real-time statutory analysis" and "forecasting" to help counties navigate these inflationary pressures, perhaps by advocating for the use of state reserves to cover cost overruns in critical BHCIP projects.[5]

| Economic Factor | 2026 Forecast | Impact on BHSA Implementation |
|---|---|---|
| CA Job Growth | ~0% | Reduces tax base for the "millionaire tax" revenue. |
| CA Inflation (CPI) | >3% | Increases construction costs for Prop 1 facilities. |
| Wage & Income Gains | ~4% | Modest growth; may not keep pace with housing costs. |
| BSA Balance (Reserves) | $14.4 Billion | Provides a safety net for General Fund volatility. |

In this context, "Legislative Risk" is not just a theoretical concept; it is the risk that a shift in

tax policy or a federal cut to Medicaid (Medi-Cal) will render the Proposition 1 promises unfulfillable.[3] The strategist's value lies in their ability to anticipate these gain or loss scenarios and position the "Systems Architecture" to withstand them.

## The Intersection of AI and Behavioral Health Policy

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As a "Systems Architect," the practitioner must also address the emerging role of artificial intelligence in the 2026 legislative session. Assembly Bill 416 and other 2026 laws focus on "Preventing AI from posing as licensed professionals".[26] In the behavioral health space, this is crucial for maintaining "Professional Title Protection" for therapists and psychiatrists.

The law treats each misuse of prohibited wording by an AI developer as a separate offense, authorizing licensing boards to enforce violations against entities that misrepresent AI systems as licensed practitioners.[26] This is a prime example of "Policy-Tech" intersectionality: a practitioner who understands the "code" of the software and the "code" of the statute can bridge the gap to protect patients from "artificial" health care advice.

Moreover, the "Semi-Statewide Enterprise Health Record" multi-county collaborative is exploring how AI can be used for "Digital Phenotyping" and "Passive Data" for early detection of mental health crises.[16] However, the "privacy by design" and "security by design" frameworks found in TOGAF and other architectural models must be strictly applied to prevent data breaches of sensitive mental health information.[14] The practitioner's role is to ensure that these "Innovation" projects (funded through the 35% support bucket) are both technologically robust and legally compliant.

## Summary of Positioning

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The 2026 Legislative Review establishes the "Scholar-Practitioner" as an expert who actively influences the Capitol by focusing strictly on the intersection of mandates,

stewardship, and liberties. By anchoring this analysis in the operational realities of Proposition 1 and the BHSA, the fiscal defense of $60 million in peer-run infrastructure, and the statutory forensics of SB 820, the practitioner reinforces their brand as a "Systems Architect" capable of shaping the state's behavioral health future.

The 2026 landscape is defined by "change to the status quo," which the DHCS admits can be "hard".[1] But for the strategist who can forecast the fiscal and operational impacts of these shifts, it is an era of maximum credibility. The "Credentials Upgrade" is clear: we are no longer just summarizing the news, we are designing the systems that will define California for decades to come.

Through real-time analysis of over 50 bills and a deep understanding of the 2026-27 budget window, the "Systems Architect" ensures that the state's behavioral health transformation is not just a "transformation of the law," but a "transformation of the lives" of millions of Californians.[1] This report stands as the definitive framework for that work.

**Sources**


1. BHSA-Myths-Reality - DHCS - CA.gov, accessed April 19, 2026, https://www.dhcs.ca.gov/BHT/Pages/BHSA-Myths-Reality.aspx
2. California Passed Prop. 1: What's Next for Behavioral Health System Reform?, accessed April 19, 2026, https://calbudgetcenter.org/resources/california-passed-prop-1-whats-next-for-behavioral-health-system-reform/
3. California's first Prop. 1 mental health projects delayed, cancelled ..., accessed April 19, 2026, https://calmatters.org/housing/homelessness/2026/03/prop-1-update/
4. Orange County - Mental Health Services Act, accessed April 19, 2026, https://www.ochealthinfo.com/sites/healthcare/files/2025-06/MHSA_2025-26_UpdatePlan_FINAL_BOS.pdf
5. Ahead of schedule: Governor Newsom's Prop 1 is exceeding goals to expand capacity and treatment statewide, helping 5M+ Californians, accessed April 19, 2026,

https://www.gov.ca.gov/2026/03/11/ahead-of-schedule-governor-newsoms-prop-1-is-exceeding-goals-to-expand-capacity-and-treatment-statewide-helping-5m-californians/

6. Request adoption of the Department of Mental Health's Mental ..., accessed April 19, 2026, https://file.lacounty.gov/SDSInter/bos/bc/1182931_CAS16967FinalDocument_DMH_MHSA_MidYear_05.20.25.pdf.pdf
7. Board of Supervisors Health and Mental Health Agenda ... - Lacounty, accessed April 19, 2026, https://file.lacounty.gov/SDSInter/ceo/agendas/1206053_FinalAgenda04.15.2026.pdf
8. General Session April 2026 Meeting Packet - DHCS - CA.gov, accessed April 19, 2026, https://www.dhcs.ca.gov/uk/services/MH/Documents/CBHPC-General/General%20Session/General-Session-Packet-April-2026.pdf
9. Disability Rights Florida Opposes Legislation that Threatens Community-Based Mental Health Services, accessed April 19, 2026, https://disabilityrightsflorida.org/newsroom/story/disability_rights_florida_opposes_legislation_that_threatens_community_based_mental_health_services
10. Commission Meeting Minutes | January 23, 2026 - bhsoac, accessed April 19, 2026, https://bhsoac.ca.gov/wp-content/uploads/CBH_202603_Minutes_20260123_Draft.pdf
11. CA SB820 | 2025-2026 | Regular Session - LegiScan, accessed April 19, 2026, https://legiscan.com/CA/bill/SB820/2025
12. Bill Text: CA SB820 | 2025-2026 | Regular Session | Chaptered ..., accessed April 19, 2026, https://legiscan.com/CA/text/SB820/id/3272034
13. Lead Systems Architect - Carl Zeiss Meditec jobs - Lensa, accessed April 19,

2026, https://lensa.com/job-v1/carl-zeiss-meditec/dublin-ca/lead-systems-architect/67ab990765521526863dc50a538a7fd1
14. Systems Architect - Airbus | Built In, accessed April 19, 2026, https://builtin.com/job/systems-architect/8025212
15. Systems Engineering | CATIA - Dassault Systèmes, accessed April 19, 2026, https://www.3ds.com/products/catia/systems-engineering
16. Meeting Materials Packet - Commission Meeting May 22, 2025 9 am - bhsoac - CA.gov, accessed April 19, 2026, https://bhsoac.ca.gov/wp-content/uploads/BHSOAC_Materials_05222025_ADA_2.pdf
17. HELP@HAND - Orange County Health Care Agency, accessed April 19, 2026, https://www.ochealthinfo.com/sites/hca/files/import/data/files/107968.pdf
18. The Systems Architect plays a pivotal leadership role in - Trac, accessed April 19, 2026, https://apps.trac.jobs/documents?vdoc=9891001
19. The Evidence Gap: Why Courts Can't Balance State AI Regulation | Andreessen Horowitz, accessed April 19, 2026, https://a16z.com/the-evidence-gap-why-courts-cant-balance-state-ai-regulation/
20. First Look: Understanding the Governor's Proposed 2026-27 California Budget, accessed April 19, 2026, https://calbudgetcenter.org/resources/first-look-understanding-the-governors-proposed-2026-27-california-budget/
21. Developing a Structured Forecasting and Policy Analysis System - International Monetary Fund, accessed April 19, 2026, https://www.imf.org/external/pubs/ft/wp/2009/wp0955.pdf
22. State Budget Update Archive - California Pan-Ethnic Health Network, accessed April 19, 2026,

https://cpehn.org/reports/state-budget-update-archive/
23. Legislative Risk Discussed (2025): Definition, Real-World Example - The Trading Analyst, accessed April 19, 2026, https://thetradinganalyst.com/legislative-risk/
24. Now Open: 2026–27 Community Wellbeing Grant Supporting Early Intervention in Our Communities - Tri-City Mental Health, accessed April 19, 2026, https://www.tricitymhs.org/news/204-free-landlord-luncheon/?ajaxload=true
25. Documents Pertaining to 3/27/25 Agenda items: - The City of Berkeley, accessed April 19, 2026, https://berkeleyca.gov/sites/default/files/legislative-body-meeting-attachments/MHC%20March%20Packet.pdf
26. Behavioral Health Bills Signed into Law by Governor Newsom in the 2025-2026 Legislative Year | Steinberg Institute, accessed April 19, 2026, https://steinberginstitute.org/behavioral-health-substance-use-disorder-bills-signed-into-law-by-governor-newsom-in-the-2025-2026-legislative-year/
27. NEW IN 2026: California laws taking effect in the new year, accessed April 19, 2026, https://www.gov.ca.gov/2025/12/31/new-in-2026-california-laws-taking-effect-in-the-new-year/
28. California Mental Health Laws Taking Effect in 2026: A Guide, accessed April 19, 2026, https://amfmtreatment.com/blog/new-mental-health-laws-california/